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TCGA-EM-A3AR-01A-PR

Redacted



Surgical Pathology Consultation Report

Patient Name:

MRN:

DOB:

Gender:

HCN:

Ordering MD:

Copy To:

Service:

Visit #:

Location:

Facility:

Accession #

Collected:

Received:

Reported:

Specimen(s) Re-

1. Thyroid: total thyroid, stitch mark left superior pole

Diagnosis

Multifocal papillary carcinoma, dominant 1.2 cm, widely invasive classical variant, left: Thyroid

Metastatic papillary thyroid carcinoma: Lymph node, 1, left

-Total thyroidectomy specimen. See Comment.

Comment

The dominant tumor is a widely invasive classical variant with multiple tumor deposits in the left lobe. There are many regions which are highly suspicious for lymphatic invasion, but there is no definitive evidence of vascular invasion. There is one lymph node on the left side which is nearly completely replaced by metastatic papillary thyroid carcinoma.

Synoptic Data

Procedure: Total thyroidectomy
 Received: Fresh
 Specimen Integrity: Intact
 Specimen Size: Right lobe:
 5.0 cm
 2.7 cm
 2.0 cm
 Left lobe:
 5.1 cm
 3.0 cm
 2.2 cm
 Isthmus +/- pyramidal lobe:
 5.5 cm
 1.5 cm
 1.3 cm
 Specimen Weight: 34.9 g
 Tumor Focality: Multifocal, bilateral
 ----- DOMINANT TUMOR -----

1CD-0-3

carcinoma, papillary, thyroid 8280/3

Site: thyroid, NOS C73.9

hw
10/21/11

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary		☒
Case In/Out	QUALIFIED	DISQUALIFIED
Reviewed Initial	10/21/11	10/22/11

Tumor Laterality: Left lobe
 Tumor Size: Greatest dimension: 1.2cm
 Additional dimension: 1.1 cm
 Additional dimension: 0.7 cm
 Histologic Type: Papillary carcinoma, classical (usual)
 Classical (papillary) architecture
 Classical cytormorphology
 Histologic Grade: Not applicable
 Margins: Margins uninvolved by carcinoma
 Distance of invasive carcinoma to closest margin: 0.1mm
 Tumor Capsule: Partially encapsulated
 Tumor Capsular Invasion: Present, extent widely invasive
 Lymph-Vascular Invasion: Indeterminate
 Perineural Invasion: Not identified
 Extrathyroidal Extension: Not identified
 ----- SECOND TUMOR -----
 Tumor Laterality: Right lobe
 Tumor Size: Greatest dimension: 0.7cm
 Histologic Type: Papillary carcinoma, classical (usual)
 Papillary carcinoma, microcarcinoma (occult, small or microscopic) variant
 Classical (papillary) architecture
 Classical cytormorphology
 Histologic Grade: Not applicable
 Margins: Margins uninvolved by carcinoma
 Tumor Capsule: Partially encapsulated
 Tumor Capsular Invasion: Present, extent minimal
 Lymph-Vascular Invasion: Not identified
 Perineural Invasion: Not identified
 Extrathyroidal Extension: Not identified
 TNM Descriptors: m (multiple primary tumors)
 Primary Tumor (pT): pT1b: Tumor more than 1 cm but not more than 2 cm in greatest dimension, limited to the thyroid
 Regional Lymph Nodes (pN): pN1a: Nodal metastases to Level VI (pretracheal, paratracheal and prelaryngeal/Delphian) lymph nodes
 Number of regional lymph nodes examined: 1
 Number of regional lymph nodes involved: 1
 Distant Metastasis (pM): Not applicable
 Additional Pathologic Findings: Other: Multiple additional papillary microcarcinomas identified in the right lobe and left lobe.

*Pathologic Staging is based on AJCC/UICC TNM, 7th Edition

Electronically verified by:
D

Gross Description

1. The specimen labeled with the patient's name and as "total thyroid stitch marks left superior pole" consists of a thyroid gland that is oriented with a suture and weighs 34.9 g. The right lobe measures 5.0 cm SI x 2.7 cm ML x 2.0 cm AP, the left lobe measures 5.1 cm SI x 3.0 cm ML x 2.2 cm AP and the isthmus measures 5.5 cm SI x 1.5 cm ML x 1.3 cm AP. The external surfaces have fibrous adhesions. No parathyroid glands and/or lymph nodes are identified grossly. The external surface is painted with Silver nitrate. The left lobe contains a well delineated nodule that measures 0.7 cm SI x 1.2 cm ML x 1.1 cm AP. The remainder of the thyroid tissue is grossly normal. Sections of the nodule and normal tissue are stored frozen. The remainder of the specimen is submitted as follows:
 A-L right lobe, superior to inferior